



Eating Disorders: Management of Patients SOP

Scope

Site	Department, division or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Medical Division, Hospital Logistics and Acute Access Division and Mental Health Division (MHD) Inpatient services, East Metro Eating Disorder Specialist Service (EMEDSS), Community Mental Health Services and Outpatient clinics	All Staff

Purpose

The purpose of this standard operational procedure (SOP) is to outline a clear pathway for the management of patients with eating disorder admitted to RPBG, at all touchpoints through the patient process, including transfers between medical and mental health services and/or discharge to community treatment programs.

For treatment and interventions refer to the [WA Eating Disorders Outreach & Consultation Service \(WAEDOCs\) Eating Disorder Management Of Youth and Adults: A Quick Reference Guide](#).

For the purpose of this document, patients and consumers across all settings will be referred to as patients.

Background

Emerging evidence suggests early intervention, including assertive nutritional rehabilitation and structured psychological support with a multidisciplinary plan for relapse prevention, are crucial for improving prognosis for patients with an eating disorder. Severe malnutrition and starvation syndrome can be experienced in people at any weight, resulting in physical and cognitive deficits including poor insight regarding nutritional and medical risk. The use of the **Mental Health Act 2014 (MHA)** may be required to facilitate safe treatment. Severely malnourished individuals are unlikely to be able to reverse starvation on their own.

Patients with high risk medical complications or recurrent presentations may require inpatient admission to enable nutritional rehabilitation and weight gain to a level sufficient for brain recovery from the atrophic changes of malnutrition, otherwise this will continue to drive eating disorder cognitions and impair engagement with psychological treatments, thereby perpetuating the cycle of recurrent weight loss and readmission. Further, starvation syndrome is associated with adverse mental health (MH) consequences which include anxiety, low mood, and suicidal ideation.

Links within document

- [Appendix I: Figure 1. RPBG Eating Disorder pathway](#)
- [Appendix II: Eating Disorders Prompt Sheet - MH Division](#)
- [Appendix III: EMEDSS response to acute physiological deterioration flowchart](#)
- [Appendix IV: A guide to applying the Mental Health Act 2014 for assessment and treatment of patients with an eating disorder](#)

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Links to relevant documents

- [Royal College of Psychiatrists Medical Emergencies in Eating Disorders \(MEED\): Guidance on recognition and management \(CR233\)](#)
- [Centre for Clinical Interventions \(CCI\) Inclusion Criteria](#)
- [Royal Australian and New Zealand College of Psychiatrists \(RANZCP\) Clinical Practice Guidelines for the Treatment of Eating Disorders \(2014\)](#)

Definitions

Acute Medical Unit (AMU)	A medical unit for acute medical and undifferentiated patients who are admitted from the Emergency Department (ED) and/or directly from other hospitals or community for assessment. The AMU has 14 Telemetry beds and 6 Health in a Virtual Environment (HIVE) beds. The AMU has a Consultant on call 24 hours a day who is available for advice and to discuss referrals for direct admission.
Eating Disorders	Are mental illnesses characterised by disturbances of eating behaviours and a core psychopathology centred on food, eating and body image concerns, e.g., Anorexia Nervosa, Bulimia Nervosa, Binge Eating Disorder, and other DSM-5 diagnoses. ¹
EMEDSS	An outpatient, specialised eating disorders service that provides care for patients 16 years+ to reduce the need for hospital (re)admission or enable early discharge from hospital. The service has a consultant psychiatrist, general practitioner (GP), psychiatry registrar, clinical nurses, a clinical psychologist, dietetics, and peer support workers.
Internal Medical Unit (IMU)	A general medicine unit for patients who require inpatient medical management for greater than the AMU timeframe of 24-72 hours. The IMU does not have cardiac monitoring facility. Note: Ward 10A does not have access to HIVE.
Consultation Liaison (C-L) Psychiatry Service	A liaison service to attend all Royal Perth Hospital (RPH) medical, surgical wards or other RPBG services whose patient cohort have a recognised increased mental illness morbidity and ensure optimal mental health assessment, treatment, and management planning.
Bentley Health Service (BHS) Safety All Hours For Everyone (BeSAFE)	Refer to SAFE and BeSAFE SOP .

Principles

The following general principles underpin the treatment for all patients with eating disorders at RPBG:

- **Multidisciplinary, coordinated care** to ensure the patient is provided a range of treatment options and a smooth transition across the continuum of care and settings including medical, dietetic, MH and other, specialist services to maximise the chances of a full recovery
- **Recovery-oriented practice** should always strive to be person centred, strengths-based and trauma informed, and is aligned with the National Recovery Framework, the MHA and the [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- **Involving families/personal support persons (PSPs)/guardians**, unless contraindicated, in the assessment and treatment process and provided with appropriate support and information
- **Least restrictive treatment** should be practiced, where possible, best suited to the patient's needs and preferences to assist with therapeutic engagement
- **A culturally informed approach to diagnosis and treatment.** Consider possible cultural and ethnic variability in the presentation of eating disorders, including sensitivity and support to the needs of Aboriginal people and other cultures, ethnicity, religions, diverse ages and individuals with sexual, gender and/or bodily diversity. ^{2, 3}

Procedure details

Refer to [Appendix I: RPBG eating disorder pathway](#) for an overview of the key pathways for the patient with an eating disorder.

The following outlines the key interphases that may be encountered by a patient who presents with an eating disorder.

Medical care interphase at RPH

Patients requiring acute medical care should be referred to a Medical Team. For patients transferred from BHS it is expected that communication from the relevant MH and Medical Teams occur in order for planning purposes and to ensure safe transfer of the patient.

Patients attending the East Metro Eating Disorders Specialist Service (EMEDSS) at Ainslie House only

For patients showing signs of acute physiological deterioration refer to [Appendix III: EMEDSS response to acute physiological deterioration flowchart](#). For information and referral form, refer to the [EMEDSS HealthPoint page](#).

Procedure details – Patients attending the EMEDSS at Ainslie House only cont'd

Table 1: Criteria for consideration of medical admission to AMU

Rapid weight loss and/or low body weight	- Loss of greater than 1 kg/week over several weeks OR - Grossly inadequate nutrition intake (less than 1000 kcal daily) for greater than 2 days OR - Body mass index (BMI) below 75% for age
Purging	Daily (uncontrolled; sufficient to cause distress and/or medical instability)
Blood Pressure (BP)	- Less than 90 mmHg systolic OR - Postural BP greater than 10 mmHg drop (lying to standing)
Heart rate	- Equal to or less than 40 beats per minute (bpm) (adolescents less than 50 bpm) or greater than 120 bpm OR - Postural tachycardia greater than 20 bpm (i.e. increase in heart rate greater than 20 bpm from lying to standing).
Electrocardiogram (ECG)	Any arrhythmia including QTc prolongation, non-specific ST or T-wave changes, including inversion or biphasic waves
Blood sugar	Below normal range less than 4.0 mmol/L
Sodium	Less than 130 mmol/L
Potassium	Below normal range*
Magnesium	Below normal range*
Phosphate	Below normal range*
Albumin	Below normal range*
Liver enzymes	Mildly elevated
Neutrophils	Less than $1.5 \times 10^9/L$
Temperature	Less than 35.5°C OR cold/blue extremities
Psychiatric concerns	Significant psychiatric risk such as deliberate self-harm or suicidal ideation. Moderate-high agitation and/or distress. These concerns are in addition to the physical parameters above which argue for a medical admission.
*to be considered in context of the overall clinical picture <i>Adapted from the WAEDOCs and RANZCP (2014) guidelines.</i>	

1. Transfer from BHS to a Medical Team at RPH: If a patient's physical status meets the criteria for a medical admission, a referral for direct admission to the AMU should be made by contacting the AMU Consultant on call/admitting Registrar for handover. **Note:** Referral age may be 16 years or over. The AMU Medical Team will provide medical advice to guide the physical management of the patient and be able to suggest the appropriate admission pathway (e.g., directly to the AMU or via ED), depending on the acuity of the patient.

The AMU admitting Registrar/Consultant on call will discuss the referral with the AMU Team who will consult with key stakeholders (e.g., RPH Operations Hub Manager, After Hours Operations Hub Coordinator, AMU Flow Coordinator and Duty Psychiatrist/Registrar) as required and communicate the outcome to the referrer in a timely manner.

It is the responsibility of the treating team to facilitate admissions to AMU in hours and Psychiatric Duty Medical Officer (DMO)/BeSAFE after hours.

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Procedure details – Patients attending the EMEDSS at Ainslie House only cont'd

2. Transfer from BHS to RPH ED: If it is considered that the patient requires urgent stabilisation for physical health deterioration, the patient should be transferred to RPH ED following discussion with BeSAFE and the RPH Emergency Registrar in Charge (ERiC) or Emergency Physician in Charge (EPiC delegate after hours).

3. Admission to AMU: On admission to the AMU, the patient should be referred immediately to the ward Dietitian (via page or MS Teams) and the RPH Consultation-Liaison (C-L) Psychiatry Service (via an e-referral). If the patient has been assessed to be medically unsafe for discharge and requires ongoing inpatient care in excess of 48 hours, the patient care will be transferred from AMU to IMU for ongoing medical stabilisation, weight restoration and concomitant psychiatric management by the C-L Psychiatry Team. If a patient under the age of 18 years is placed on an Inpatient Treatment Order as per the MHA the patient must be specialised when admitted to an adult ward. Refer to the **Special and Supportive Observations Policy**.

In circumstances that warrant a chaperone, refer to the **EMHS Chaperone Policy**.

4. Transfer between RPH and BHS: all MH patients with comorbid eating disorder referred for medical stabilisation from BHS should be discussed with the referring treating Psychiatrist before discharge/transfer decisions are made. Consultant to Consultant discussions will decide if discharge or transfer back to an inpatient MH bed is appropriate. **The patient must not meet any of the exclusion criteria listed in [Table 1](#).** Repatriation to a MH bed will be prioritised if further specialist psychiatric management is required.

RPBG Consultation-Liaison (C-L) Psychiatry Service

The C-L Psychiatry Service will be referred patients with an eating disorder patients by the AMU Team as soon as they are admitted to the medical unit by e-referral. The C-L Psychiatry Service will remain involved in the patient's MH management for the duration of their admission at RPH and may consider referral to EMEDSS.

The C-L Psychiatry Service will play an integral role in the discharge planning for patients with eating disorders, providing advice to the Medical Treating Teams regarding transfer to MH units, as either a voluntary or involuntary patient, or assist the Medical Team to identify suitable community-based services, once patient is deemed medically stable. Refer to [Appendix IV: A guide to applying the MHA 2014](#).²

Procedure details cont'd

Medical inpatient treatment/management (i.e., AMU/IMU)

All patients identified with an eating disorder that require medical management will be admitted in the first instance to the AMU when not requiring a critical care bed e.g., intensive care unit (ICU)/high dependency unit (HCU). Patients requiring urgent medical stabilisation will be admitted via ED. If patients require a length of stay (LoS) greater than 48 hours then patient care will be transferred to the IMU for further management.

The AMU and IMU clinicians, in liaison with Dietetics, will assess and manage any risk of refeeding syndrome, and address any complications related to malnutrition. Refer to the **Refeeding Syndrome Clinical Guideline** for management. The patient will receive ongoing Dietitian review and their nutritional management will be guided by the WAEDOCs guidelines.

MH assessment via the C-L Psychiatry Service is an essential aspect of the management of patients with an eating disorder.

The goals of medical management aim for medical stabilisation via:

- Prevention and treatment of re-feeding syndrome
- Safe nutritional and weight restoration
- Reversal of acute cognitive effects of starvation.

The following are the key aspects to medical management:

Daily observations

- U+E, Mg²⁺, PO₄, and full blood picture (FBP)
- Liver function test (LFT), B12 / Folate / Ca²⁺ (on admission and then as clinically indicated)
- Other investigations as indicated by clinical findings
- QID observations of:
 - Temperature
 - Respiratory Rate
 - Lying and standing heart rate, **and** BP – if safe to do so
- Daily ECG, refer to **12 Lead Electrocardiograph (ECG) Patient Management Clinical Practice Standard (CPS)**
- Blood glucose levels (BGLs) QID (2 hours post prandial irrespective of feeding route) and 02:00 hours
- Fluid balance chart
- Dietary intake chart, from commencement of oral intake
- Daily weights at 06:00 hours, post void and in a hospital gown

Daily supplementation

- 300 mg thiamine OD
- 1x multivitamin and mineral supplement OD
- 1x B complex supplement OD
- 500 mg phosphate Sandoz BD (unless the level is elevated)
- Replace electrolytes as clinically indicated.

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Procedure details – Medical inpatient treatment / management (i.e., AMU/IMU) cont'd

Escalation, as required

Notify the Resident Medical Officer (RMO) if:

Code Blue/Medical Emergency Team (MET) Call has been activated as per criteria or if any of the following:

- Pulse is below 50 bpm confirmed 50 bpm correct for this cohort
- Postural drop of more than 10 mmHg or:
- Postural tachycardia greater than 20 bpm
- QTc interval greater than 450 ms
- BGL less than 4.0 mmol/L

Refer to **Medical Emergencies and Cardiorespiratory Arrest Management CPS**.

Determination of medical stability is guided by the following parameters:

1. Cardiovascular stability and biochemical indices continuing to trend in the normal ranges
2. For patients with anorexia nervosa, a BMI of greater than 14.0 kg/m² is a guide to transferring an ED patient from a general hospital to a MH unit for optimal treatment of the patient's mental illness and further nutritional stabilisation. For patients with other disorders of eating, the target BMI and weight trajectory for medical stability will be determined by treating team that jointly consists of the Medical, Dietetics and C-L Psychiatry Team.

Once the patient has been assessed as medically stable for a period determined by the patient's treating Physician; the Physician, in liaison with the RPH C-L Psychiatry Service, will contact the patient's MH Team to facilitate transfer to a MH inpatient unit. This discussion will occur at a Consultant to Consultant level. Consider application of the MHA for patients who lack capacity prior to transfer to a MH inpatient setting (refer to [Appendix IV](#)).

Transfer to a mental health inpatient unit

Weight restoration and nutrition is managed by the ward-based Dietitian in MH inpatient units. Route of nutrition restoration will be dependent on medical stability and BMI, and this may be enteral or oral nutrition, depending on the resources.

Note: Currently, BHS has limited capacity to provide naso-gastric tube (NGT) nutrition to MH services and is to be considered on a case-by-case basis dependent on ward acuity and available resources. BeSAFE are available to assist in patient care, i.e., insertion and troubleshooting of NGT and for escalation of physical health concerns in the after hours space.

MH areas do not have capacity to mechanically restrain patients, therefore, patients who require the use of mechanical /physical restraint for NGT feeds will not be able to be managed at BHS. Patients who require physical restraint for NGT insertion may require transfer to RPH ED for insertion, prior to transfer back to BHS for continuation of treatment.

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Procedure details – Transfer to a mental health inpatient unit cont'd

All transfers to MH inpatient units of patients with eating disorder comorbidity, must include completion of a risk assessment prior to transfer, to determine safe capacity and appropriate management. The number of such patients across all RPBG wards will be determined on a case by case basis. A Prompt Sheet ([Appendix II](#)), to be used by the referring clinician, is available from the MH service Bed Manager for a patient with an eating disorder referred to MHD inpatient units and is filed in the correspondence section of the patient's medical record, with other, referring documentation. This ensures all relevant information is available to assist decision making to determine suitability for management in a peripheral hospital setting. The primary responsibility for overall patient care remains with the current treating team.

If a patient is no longer at risk of refeeding syndrome and considered medically stable by the treating Medical Team, then transfer to a MH unit will be considered on a case by case basis. The criteria outlined below can be used to guide medical clearance for transfer, in conjunction with discussion between all members of the MDT including Medical, Psychiatry and Dietetics.

The treating medical team, in conjunction with the C-L Psychiatry Service, will assess each patient regarding their suitability for transfer to a MH unit with the consideration of the following measures.

Weekends/after business hours referrals/admissions

Patients who meet the following criteria are **only** to be admitted by the patient's treating Physician during regular East Metropolitan Youth Unit (EMyU) team/business hours (i.e., not after hours):

- Presentations needing daily Dietician input
- Patients who do not have a meal plan
- Patients without 100% compliance of their meal plan in the last 48 hours and
- Anything outside the normal criteria for EMyU admission

Consider transfer to a mental health service if medically stable* as per:

- BMI greater than 14 kg/m².
 - **Note:** For EMyU, a BMI equal to or greater than 16 kg/m² is the minimum accepted for transfer unless Consultant approval for a lower BMI.
- Systolic BP 90 mmHg or above (**or** in the case of systolic BP between 80 mmHg and 90 mmHg, if reviewed and agreed in conjunction with Psychiatrist in charge of the MH service)
- Heart rate between 50 bpm and 100 bpm
- No significant postural tachycardia
- Normal ECG
- Normal electrolytes
- Stable BGLs that are below the normal range.

This will be determined by treating medical team in liaison with C-L Psychiatry and Dietetics. The C-L Psychiatry Service will provide advice regarding MH issues that require further management in an inpatient setting to the receiving MH service.

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Procedure details – Transfer to a mental health inpatient unit cont'd

Consider patient discharge to the community if medically stable, PLUS:

- Consuming regular adequate nutrition on and off the ward. Monitoring of this is to include consultation with the Treating team/ patient and carer/PSP/guardian
- Leave entitlements (if appropriate) have been successfully followed without deterioration in medical stability. **Note:** IMU inpatients are not eligible for overnight leave.
- Cognitive effects of starvation have reduced sufficiently for the patient to be able to engage in outpatient psychotherapy (may need BMI greater than 16 kg/m²), for e.g.:
 - Impaired concentration, judgement and decision making
 - Impaired comprehension
 - Increased rigidity and obsessional thinking
 - Reduced alertness.
- C-L Psychiatry have been involved and approve of discharge to community
- The relevant referring MH inpatient unit has been consulted in the discharge planning process.

Community management

EMEDSS

- EMEDSS can receive referrals by a Psychiatrist or Psychiatric Registrar from inpatient medical wards, MH wards and community MH teams
- The EMEDSS team will:
 - Support patients who have recently been discharged from an inpatient setting, and those at risk of requiring admission due to an eating disorder
 - Provide guidance on management and treatment goals for existing patients who require readmission
 - Complete comprehensive triage
 - Ensure patient transition coordination.

Inclusion criteria

Patients must:

- Be aged 16 years and over
- Reside within the EMHS catchment area
- Have an eating disorder as diagnosis of primary concern
- Actively and regularly be engaged with a GP in the community, or willing to do so
- Be medically stable in accordance with WAEDOCS guidelines*

* Consideration for patients with measures outside the WAEDOCS guidelines will be considered on a case-by-case bases in consultation with the EMEDSS Consultant Psychiatrist or Consultant Physician.

Procedure details – Community management cont'd

Exclusion criteria

Patients who:

- Are living outside the EMHS catchment area
- Do not have a diagnosis of an eating disorder
- Have current psychosis
- Have active substance use
- Have current medical instability
- Have acute psychiatric risk (currently acutely suicidal or significant deliberate self-harm requiring medical attention)
- **Intensive Clinical Monitoring (ICM).** The nurse and Peer Support Worker are to:
 - Provide nutritional and physical health status monitoring and motivational support
 - Offer up to 4 appointments per week for a maximum of 8 weeks, as determined by clinical need
 - Problem-solve any patient concerns and encourage to maintain progress
 - Complete physical observations on *Adult Observation and Response Chart (AORC)*
 - For escalation of care refer to guidance in [Appendix III](#), ensure BGLs on x, ECGs, and bloods (off-site) as required, nutritional monitoring via food record charts.
- **Specialist Multidisciplinary Outpatient Clinic**
 - Is time limited for a period of up to 3 months to allow patients to transition to suitable community services
 - The MDT will be available to provide in-reach transition support, relapse prevention planning, physical health assessment, evidenced- based therapies and support for general health and wellbeing.

Community Mental Health Teams (CMHS)

Patients who are referred to RPBG CMHS with eating disorders are assessed and treated on a case by case basis. There are no specialised programs provided. CMHS staff can provide additional support for clients with co-occurring or complex MH needs who are also attending EMEDSS.

There is a requirement that all patients with eating disorders have their physical health monitored and managed by a GP/EMEDSS. In addition, psychological and dietetics input is encouraged to be provided via **external services** given the limited resources for this expertise across the CMHS.

Community resources may be accessed – e.g.:

- Centre for Clinical Interventions (CCI) eating disorder outpatient program
- Private hospital inpatient programs
- Swan Centre
- Freedom Co.
- The Body Esteem Program.

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Compliance monitoring

- Quarterly review by the MHD, RPBG Medical Clinical Lead and Lead C-L Psychiatrist of specific cases with key stakeholders to identify compliance, effectiveness and gaps
- Clinical incidents reported in Datix Clinical Incident Management System (CIMS)
- RPBG Performance Monitoring Schedule reported by MHD quality and safety meetings.

Facilitators

Marika Spaseska, Policy Officer, Innovation and Education, RPBG

Jude Cuff, Divisional Safety & Quality Coordinator (S & QC), Hospital Logistics & Acute Access (HoLAA) Division, RPBG

Lee Hanlon-Morris, S & QC, Medical Division, RPBG

Jessica Casado, S & QC, Surgical Division, RPBG

Review authors

Jason Ellis, Deputy Director, MHD, RPBG

Dr Vinesh Gupta, Medical Co-Director, MHD, RPBG

Dr Divya Ganapathy Rao, Consultant Psychiatrist Clinical Lead, EMyU, MHD, RPBG

Dr Matthew Leung, Psychiatry Service Registrar, EMyU, MHD, RPBG

Sarah Galbraith, NUM, EMyU, MHD, RPBG

Alexandra McGlynn, Coordinator of Nursing, HoLAA, RPBG

Stephanie Berry, A/NUM, Acute Medical Unit, RPBG

Linda Tierney, Service Manager, EMEDSS, EMHS

Rebecca Ng, Consultant Psychiatrist, EMEDSS

Fintan O'Looney, CNC, EMEDSS

Stephanie Mattys, CNS, EMEDSS

David Gray, NUM, Ward 6G, Department of General Surgery, RPBG

Wayne Epton, Head of Department, Department of Nutrition and Dietetics, RPBG

EMEDSS Flowchart endorsed by RRAD Committee 22/08/2023

Aboriginal Health Strategy Team, East Metropolitan Health Service

Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Eating Disorders: Management of Patients SOP through consultation with Aboriginal Health Strategy Team and the Aboriginal Workforce Engagement Group which includes all Aboriginal staff (clinical and non-clinical) across EMHS including the Aboriginal Health Liaison Officers, Aboriginal Health Strategy Team, and Aboriginal Acute Care Coordination Team to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 3007).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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Related policies, practice standards, clinical guidelines

R PBG Policy Hub

- [Recognising and Responding to Acute Deterioration Policy](#)
- [Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP](#)
- [Nutrition and Hydration CPS](#)
- [Enteral Tubes and Enteral Feeding Administration and Management CPS](#)
- [Clinical Handover Policy](#)
- [SAFE and ^{Be}SAFE SOP](#)
- [Special and Supportive Observations Policy](#)
- [Refeeding Syndrome Clinical Guideline](#)
- [12 Lead Electrocardiograph \(ECG\) Patient Management Clinical Practice Standard \(CPS\)](#)
- [Medical Emergencies and Cardiorespiratory Arrest Management CPS](#)
- [Behaviours of Concern \(Preventing, Recognising and Responding\) Practice Standard](#)
- [Minimisation and Safe Management of Restraint and Seclusion \(for Mental Health Areas\) CPS](#)

EMHS Policy Hub

- [Acute Response Mental Health Emergencies/Crisis Policy](#)
- [Care Coordination in Mental Health Services Policy](#)
- [Chaperone Policy](#)

Department of Health Policy Hub

- [Safety Planning for Mental Health Consumers Policy](#)
- [Emergency Departments Policy](#)
- [Statewide Standardised Clinical Documentation \(SSCD\) for Mental Health Services Policy](#)

Other

- [Mental Health Act 2014](#)
- [Chief Psychiatrist's Standards for Clinical Care 2015](#)
- [WA Eating Disorders Outreach & Consultation Service \(WAEDOCS\) Eating Disorder Management Of Youth and Adults-A Quick Reference Guide](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)

Legislative requirements, the evidence, and the standard

For information relating to the legislative requirements and standards that R PBG policy documents must adhere to, and regarding the logos and levels of evidence used within R PBG policy documents, refer to [Legislative Requirements, the Evidence and the Standard](#) (live link) on the Policy HUB.

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Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021)

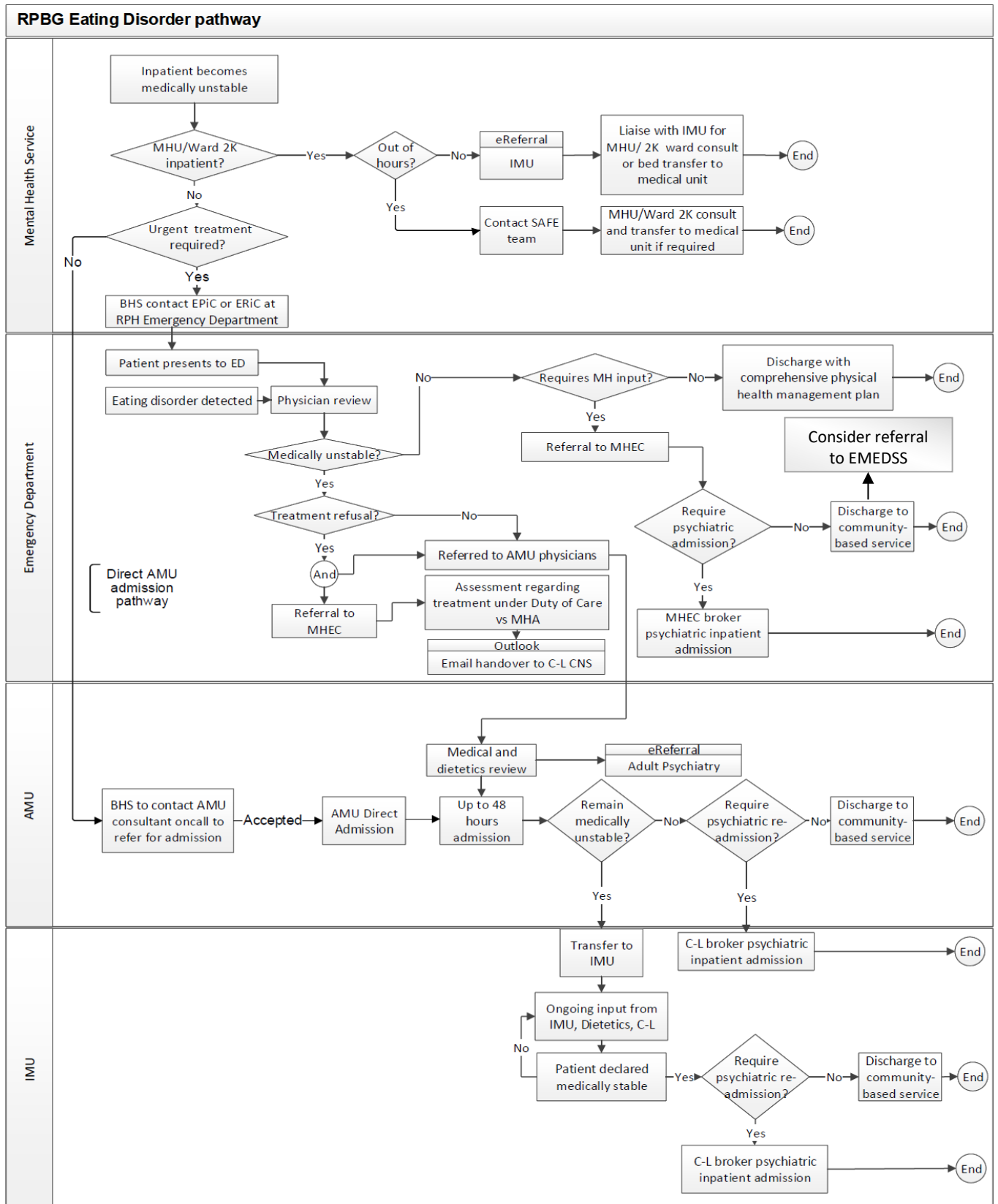
Standard 5: Comprehensive Care; Standard 8: Recognising and Responding to Acute Deterioration

NSMHS Standards (2010): Standard 2: Safety; Standard 6: Consumers; Standard 10: Delivery of Care

References

1. American Psychiatric Association. (2013). *Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition (DSM-5)*. Washington, DC: American Psychiatric Press.
2. State of Queensland (Metro North Hospital and Health Service). (2020). *Guide to Admission and Inpatient Treatment*. Metro North Queensland Eating Disorder Service (QuEDS). Queensland.
3. Royal Australian and New Zealand College of Psychiatrists (RANZCP). (2014). *RANZCP clinical practice guidelines for the treatment of eating disorders*. Australian and New Zealand Journal of Psychiatry, Vol. 48 (11) 1-62. New South Wales. Australia.

Appendix I: RPBG eating disorder pathway



LEGEND

- Internal Medical Unit (IMU)
- Acute Medical Unit (AMU)
- Mental Health Unit (MHU)
- Safety After Hours For Everyone (SAFE)
- RPH Emergency Registrar in Charge (ERIC)
- Mental Health Emergency Centre (MHEC)
- Consultation Liaison (C-L) Psychiatry Service
- Royal Perth Hospital (RPH) Emergency Physician in Charge (EPiC)
- East Metro Eating Disorder Specialist Service (EMEDSS)
- MH: Mental Health
- Mental Health Act 2014 (MHA)
- Bentley Health Service (BHS)

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Appendix II: Eating Disorders Prompt Sheet - Mental Health Division (MHD)

Patient Name:	D.O.B.:	UMRN:
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For referrals of patients with a history of restricted eating or purging behaviour, the MHD Inpatient areas require the following to be completed:

(1) TICK ONE OF THE BELOW:

- 16- to 17-year-old EMHS or NMHS catchment ☐
- 16- to 17-year-old SMHS catchment ☐ – **refer to FSH youth unit (FSH MHB) first**
- 18 years+ EMHS catchment ☐
- Adult out-of-area catchment (NMHS metro/rural or SMHS metro/rural) ☐ – **exclusion criteria listed in [Table 1](#) in the Eating Disorders: Management of Patients SOP – admission requires authorisation by MH Inpatient Consultant Psychiatrist**

(2) REQUIREMENTS FOR REFERRAL BEFORE PRESENTING TO MENTAL HEALTH (MH) Inpatient Area/ Duty Medical Officer:

- All standard requirements for admission (see overleaf) (**all referrals**) ☐
- Eating history (**all referrals to provide**) supporting information to indicate the below 2a-c requirements):
 - Duration of restriction/purging ☐
 - Degree of restriction/purging, including any weight loss ☐
 - Presence or absence of previous diagnoses ☐
- WAEDOCS screen (**all referrals**):
 - Postural heart rate ☐
 - Postural blood pressure ☐
 - Blood sugar level ☐
 - ECG ☐
 - Weight ☐
 - BMI ☐
 - Potassium ☐
 - Magnesium ☐
 - Phosphate ☐
- IF** prolonged stay in ED **OR** admission to medical ward:
 - Intake chart ☐
- IF** admission to medical ward:
 - Meal plan ☐
 - Notes from dietetics review ☐
- Does the patient need ongoing meal support (extra nursing care)? (**all referrals**)
 - YES ☐ **OR** NO ☐

If clinically significant abnormalities on WAEDOCS screen (see criteria overleaf), ensure documentation that the emergency department (ED) / medical team deem this manageable on a mental health ward/unit.

Appendix II: Eating Disorders Prompt Sheet - Mental Health Division cont'd

Royal Perth Bentley Group

Table 1: Criteria for consideration of medical admission

Rapid weight loss and/or low body weight	- Loss of > 1kg/week over several weeks OR - Grossly inadequate nutrition intake (<1000kcal daily) for > 2 days OR - Body mass index (BMI) below 75% for age
Purging	Daily (uncontrolled; sufficient to cause distress and/or medical instability)
Blood Pressure	- <90mmHg systolic OR - postural blood pressure >10 mmHg drop (lying to standing)
Heart rate	- ≤40bpm (adolescents <50 bpm) or >120bpm OR - postural tachycardia >20 bpm (i.e. increase in heart rate >20 bpm from lying to standing)
Electrocardiogram (ECG)	Any arrhythmia including QTc prolongation, non-specific ST or T-wave changes, including inversion or biphasic waves
Blood sugar	Below normal range < 4.0mmol/L
Sodium	<130mmol/L
Potassium	Below normal range*
Magnesium	Below normal range*
Phosphate	Below normal range*
Albumin	Below normal range*
Liver enzymes	Mildly elevated
Neutrophils	<1.5 x 10 ⁹ /L
Temperature	<35.5°C OR cold/blue extremities
Psychiatric concerns	Significant psychiatric risk such as deliberate self-harm or suicidal ideation. Moderate-high agitation and/or distress. These concerns are in addition to the physical parameters above which argue for a medical admission.
*to be considered in context of the overall clinical picture Adapted from the Western Australian Eating Disorder Outreach & Consultation Service (WAEDOCS) and RANZCP (2014) guidelines.	

Taken from RPBG's Eating Disorders: Management of Patients SOP

Standard requirements for admission to MH Inpatient Area:

1. 8-page Mental Health Assessment Form or 12-page CAMHS Assessment, or the same history presented in an alternative format (e.g. MH Triage or in a letter). Must include:
 - a. history of presenting complaint
 - b. past mental health (MH) and medical history
 - c. mental state examination including formulation and plan
2. Current WA Hospital Medication Chart (HMC) (or a complete and up-to-date medication list)
3. Documented that patient is assessed and medically stable for MH admission. **This must address any clinically significant abnormalities identified on WAEDOCS screen**
4. Completed screening upon presentation as per the [Respiratory Virus Illness Prevention and Management Clinical Guideline](#)
5. Risk Assessment and Management Plan (RAMP)
6. Mental Health Act 2014 forms, if applicable

NB: Prompt Sheet is available from the MH Service Bed Manager for a patient with an eating disorder referred to MHD inpatient units and filed in the correspondence section of the patient's medical record

Eating Disorders: Management of Patients SOP

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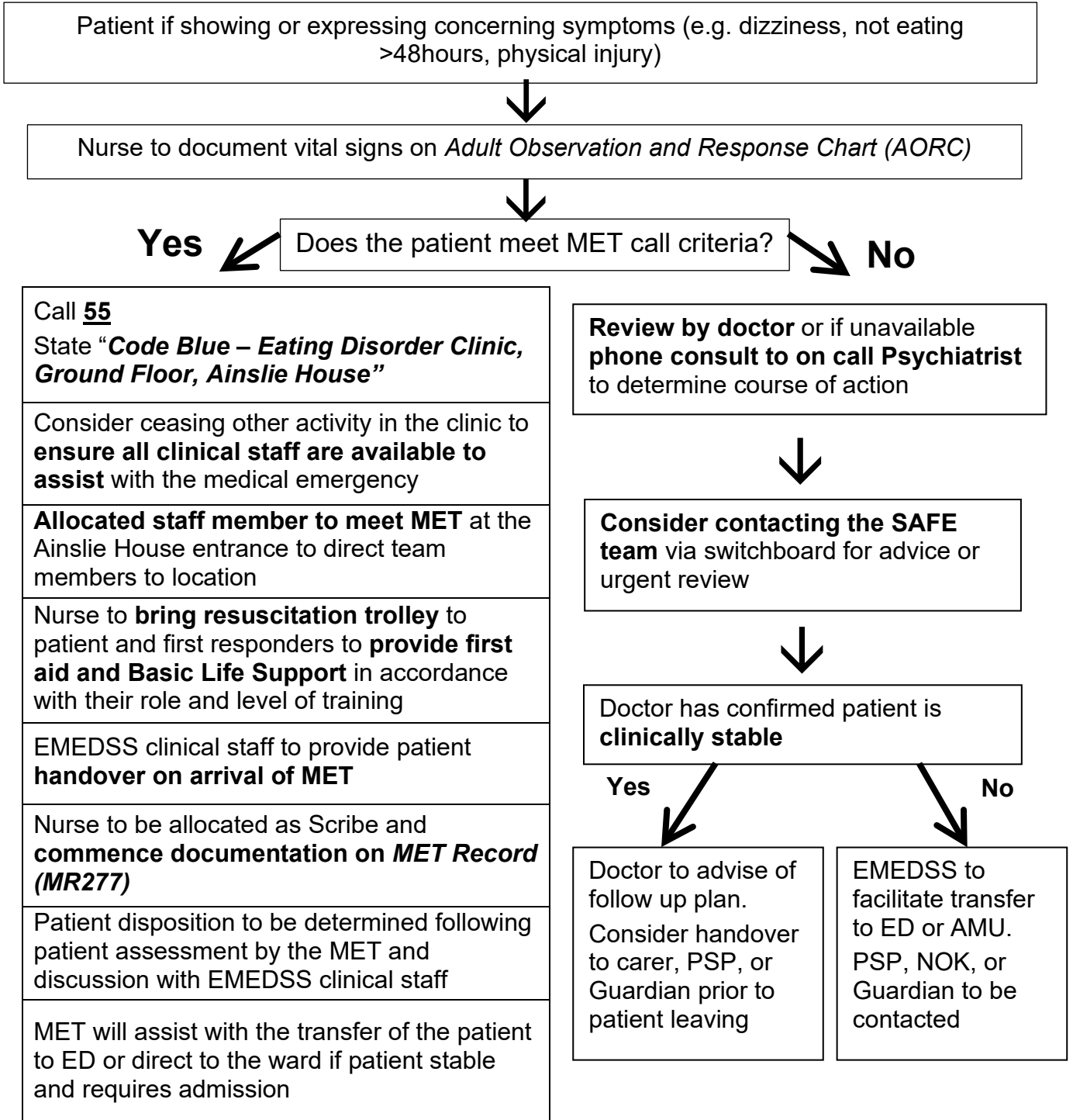
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Appendix III: EMEDESS response to acute physiological deterioration flowchart

EMEDSS CLINIC (AINSLIE HOUSE) RESPONSE TO ACUTE PHYSIOLOGICAL DETERIORATION



Appendix III: EMEDESS response to acute physiological deterioration flowchart cont'd

NB: Only applicable to the EMEDSS while operating out of Ainslie House.



Royal Perth Bentley Group

MET

(Medical Emergency Team)

Calling Criteria

For all conditions listed below
Call MET
Phone 55 state “**code blue medical emergency**”
and identify location

Acute changes in:	Physiology
Airway	Threatened
Breathing	Respiratory rate ≤ 4 Respiratory rate ≥ 36 O ₂ Saturation $\leq 84\%$
Circulation	Pulse rate < 40 Pulse rate > 140 Systolic blood pressure < 90
Neurology	Sudden fall in level of consciousness (Fall in GCS of > 2 points) Repeated or prolonged seizures
Other	Any patient who you (or a family member/carer) are seriously concerned about that does not fit the above criteria

For all cardiac and respiratory arrests
Phone 55 immediately
State “code blue cardiac arrest” and identify location

Refer to

- **Medical Emergency and Cardiorespiratory Arrest Management CPS**
- **SAFE and BeSAFE SOP**
- [EMEDESS HealthPoint page](#)

Appendix IV: A guide to applying the Mental Health Act 2014 for assessment and treatment of patients with an eating disorder

Using the Mental Health Act (MHA) 2014

Eating Disorders are mental illnesses that can be life-threatening and associated with impaired capacity due to the mental illness itself as well as the physical effects of starvation on brain. The MHA can be used where the person's impaired capacity is putting them at risk and there is no less restrictive way of ensuring they receive treatment. Below is a list of the criteria for involuntary treatment under s25 of the MHA and notes to assist in deciding whether the criteria may apply to a patient with an eating disorder. To justify involuntary treatment, all criteria (a) to (d) must apply:

a) The person has a mental illness

Note: Anorexia Nervosa, Bulimia Nervosa, Other Specified Feeding or Eating Disorder (previously Eating Disorder Not Otherwise Specified), Avoidant Restrictive Food Intake Disorder (ARFID) and Un-specified Feeding or Eating Disorder are all listed as mental illnesses in DSM-V.¹

b) The person does not have capacity to consent to be treated for the illness

Note: In addition to the capacity-impairing symptoms of eating disorders such as intense fear of eating and/or gaining weight, starvation of the brain can cause profound cognitive changes such as rigid, inflexible thinking, obsessive preoccupation and fear around food and weight, ritualised behaviours around food and exercise, depression, and extreme decision-making difficulty. These capacity-impairing effects of starvation can be often reversed with nutrition. For more information, see *The Biology of Human Starvation* by Keys, Henschel & Brožek or, for a summary of the study go to [The Great Starvation Experiment, 1944-1945](#) or [The Effects of Starvation - Inside Out Institute](#).

(c) Because of the person's illness, the absence of involuntary treatment is likely to result in imminent serious harm or serious mental or physical deterioration

Note: Anorexia Nervosa has the highest mortality rate (up to 20%) of any mental illness (See [About Eating Disorders - InsideOut Institute](#), 2023). Deaths can be due to reversible complications of malnutrition, eating disorder behaviours, and patient suicide. The Minnesota semi-starvation study demonstrated that starvation causes predictable mental, behavioural and physical symptoms that only reverse with nutritional rehabilitation (Refer to [The Great Starvation Experiment, 1944-1945](#).)

(d) There is no less restrictive way for the person to receive treatment and care for their mental illness

Note: refer to admission and treatment considerations (see [Table 1](#) of this document, page 4).

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Appendix IV: A Guide to Applying the Mental Health Act 2014 for Assessment and Treatment of Patients with an Eating Disorder cont'd

Use of restrictive interventions

In determining whether a person is to be treated involuntarily under the MHA, clinicians must use the principles of trauma-informed care and consider least restrictive principles in providing treatment and care. An authorised doctor may make a Treatment order if treatment is considered necessary and no less restrictive options are appropriate to meet the person's needs. If involuntary treatment or care under the MHA is considered necessary in a medical ward, it is advisable to contact the hospital C-L Psychiatry Team to seek assistance regarding the use of the MHA and discuss with the person and/or family/carers, if clinically appropriate. Guidance on less restrictive ways and resources can be accessed via **RFBG Prevention and Management of Aggressive Behaviours within MH Inpatient Setting SOP** or **Behaviours of Concern CPS** or the [Chief Psychiatrist of Western Australia website](#).

Use of seclusion and restraint

Seclusion and physical restraint are to be used only as a last resort, where less restrictive interventions have been unsuccessful or are not feasible and according to the provisions of the MHA and the Chief Psychiatrist's Standards of Care, unless authorised under another law. For further information refer to the [Chief Psychiatrist of Western Australia website](#) and the **RFBG Minimisation and Safe Management of Restraint and Seclusion (for Mental Health Areas) CPS**.

Guardianship and Administration Act 1990 (GAA)

If the person with an eating disorder has impaired capacity to provide or withhold consent to treatment, treatment without consent may also be possible under the GAA, if the criteria within the legislation are met.

*Adapted from the [Queensland Eating Disorder Service \(QuEDS\) Guide to Admission and Inpatient Treatment \(2020\)](#)

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